FOR DR ODEDINA, MEDICAL DIRECTOR

What we need from you through this programme

The places where the Medical Director is the only person who can act

Date 18 September 2026
From Dr Debo Odulana, Founding Partner, Consult for Africa
Purpose Answering the question you asked: how you participate, what we need, and how much of your time it takes

Dear Dr Odedina,

Thank you for asking the question, and thank you for the way you asked it, because the honest answer is that this programme needs more from you than from anyone else on the board and I would rather set that out plainly now than have it arrive as a series of requests over the next six months.

The reason is straightforward. The audit rated clinical governance at 1.5 out of 5, which is the lowest rating anywhere in the report, and clinical governance is the one area that cannot be delegated to operations, to a general manager, or to us, because it requires a doctor to sign it and a Medical Director to own it. Separately, the growth plan is led by the neonatal unit, and neonatal referral is a clinical conversation between consultants rather than a marketing exercise, so the commercial plan also runs through you. Those two facts between them define everything below.

There are five things I want to say plainly up front.

The first is clinical governance, and it is the priority

Haven has no clinical incident register, no clinical audit programme, and no clinical governance committee, so when something goes wrong it is handled verbally and well by the people present, and then it leaves no record, which means the hospital cannot learn from it, cannot show a regulator or an insurer how it manages clinical risk, and cannot defend itself if it is ever challenged. Your team is already willing to report, which is the hard part and the part most hospitals never achieve, so what is missing is only the instrument and the person who owns it.

What we need from you is to chair a clinical governance committee once a month, to own the incident register and review it weekly for fifteen minutes with nursing and pharmacy present, and to approve a rolling clinical audit plan that we will draft for you. We can build all of it, and we will, but it has to carry your signature and your chairmanship or it is not clinical governance at all.

The second is the clinical standards that are waiting for a signature

Several documents need a Medical Director to approve them and currently do not have one. The transfusion pathway is the most pressing, because the laboratory procedure is titled to include crossmatch and requires a crossmatch release form on every unit issued, yet it contains no crossmatch method, and the nursing procedure sets out a full transfusion procedure and then instructs nurses not to perform transfusions, and it directs consent to be taken from the doctor or the laboratory scientist rather than from the parent. Transfusions are being performed, so this needs rewriting as one document with the compatibility method stated, consent assigned to the parent, and the person who administers named.

Alongside it sit the critical value reporting pathway, which does not exist, the paediatric age-banded reference ranges, which the laboratory issues results without, and the resuscitation currency register, since recent resuscitation training scored 2.80 out of 5 and is the weakest single item in the staff survey. Both nursing procedures are past their review date and the pharmacy procedure carries no named approver at all. We will draft each of these. You review, amend and sign.

The third is the neonatal referral conversation, which only you can hold

The neonatal unit is the highest-yield service Haven operates at roughly three million naira per admission across three cots, and it is materially under-used, so filling it is the single largest commercial lever in the plan. Our business development team works genuinely hard and is not the constraint, but neonatal referral is won with obstetricians, paediatricians and maternity centre operators who want to know cot capability, admission criteria, what Haven will and will not accept, how transfer works, how escalation works, and what happens to their relationship with the family afterwards, and that is a consultant-to-consultant conversation.

What we need is for you to attend the referral meetings that matter, to host obstetricians on a tour of the unit so they can see the capability for themselves, and to design an antenatal pathway with them so that high-risk pregnancies are co-planned and the babies who will need a cot are effectively pre-booked. We are not asking you to sell. We are asking you to be the clinical authority in the room while somebody else sells.

The fourth is the time this actually takes

I would rather give you a real number than an encouraging one. The governance work is roughly one half-day a month for the committee plus fifteen minutes a week on the incident register. The standards work is front-loaded and heavier, at perhaps two half-days over the first six weeks to review and sign what we draft, and then very little. The referral work is the largest and the most valuable, at around two clinician visits or tours a month through the high season from November to February. Taken together that is close to two days a month, weighted towards the first six weeks and towards the referral programme thereafter.

The fifth is the survey, which is still outstanding

Four of the five founders have completed the direction survey and you are the one outstanding, which matters more than it might appear, because the four answers so far split two against two on how ambitious Haven should be over the next two to three years and split two against two again on whether to build functions in-house or outsource them. You are the casting vote on both, so the synthesis we bring to the strategy session is materially weaker without you. It takes about twenty minutes and the link is in the email this note came with.

> **In one line: we need you to own clinical governance, sign the clinical standards, and be the clinical face of the neonatal referral programme. Everything else we can carry.**

A summary you can work from

What	Your part	Our part	When
Clinical governance committee	Chair it monthly	Draft the terms of reference, the agenda and the papers	First meeting in October
Clinical incident register	Own it, review weekly for fifteen minutes	Build the form, the register and the corrective action log	Live within two weeks
Transfusion pathway	Review, amend and sign	Rewrite it as a single document across laboratory and nursing	Draft to you in two weeks
Critical values, paediatric ranges, resuscitation currency	Approve and set the standard	Draft each one and build the currency register	Through October

What	Your part	Our part	When
Clinical audit programme	Approve the annual plan, chair the findings review	Draft the plan and run the first cycle with your team	Plan in October, first audit in November
Neonatal referral programme	Attend the meetings that matter, host unit tours	Build the target list, the clinical referral pack and the tracking	From October, heaviest November to February
Antenatal pathway with obstetricians	Design it clinically with them	Set up the relationships and document the pathway	November onward
Founders' direction survey	Twenty minutes, on your own	Synthesise all five and bring the pattern to the session	Before the strategy session

None of this asks you to run the hospital day to day, and in fact the whole board answered unanimously in the survey that daily management should pass to professionals, which is the recommendation we have made and which the senior operations appointment is meant to deliver. What it does ask is that the clinical standard of the institution has a named owner who is a doctor, because at the moment it does not, and that is the gap the audit found.

The natural first step is a conversation between the two of us before the strategy session, so that you can push back on any of this while it is still a draft. I am happy to come to you.

Warm regards,

Dr Debo Odulana

Founding Partner, Consult for Africa